



Armada Kalamunda Group

Non-Specific Abdominal Pain in Pregnancy >20 weeks Guideline

1. Purpose

The initial goal is to identify woman who have a serious or even life-threatening aetiology for their symptoms and require urgent intervention¹.

Additional issues in pregnant women include consideration of the impact of physiologic changes related to pregnancy, causes of acute abdominal/pelvic pain that may be more common due to the pregnant state or related to obstetrical complications, and the impact of the disease process on the fetus

2. Applicability

This guideline applies to medical and midwifery staff working within the Maternity Unit at Armada Kalamunda Group.

3. Background

Non-specific abdominal pain in pregnancy may not typically present as obstetric in nature, but conditions such as threatened premature labour, pre-eclampsia and placental abruption may have atypical presentation at early gestation.

Other conditions such as urogenital infection, or other pelvic / abdominal inflammation / infections may lead to preterm labour.

4. Assessment:

All assessment should be documented ideally on AKMR 8: Maternal Assessment form.

All women presenting with abdominal pain must be reviewed by on call Obstetrician (GPO) or specialist after the initial assessment. All pregnant women ≥20 weeks' gestation should be assessed in AAU after initial assessment in ED and if deemed safe to be sent to AAU:

- Document the history of abdominal pain noting:
 - The duration and nature of pain e.g., crampy, continuous, intermittent
 - Site of pain
 - Any urinary and bowel symptoms
 - The presence of any vaginal bleeding or discharge
 - Any precipitating event
- Document any relevant present and past history.

- Document presence and nature of fetal movements. Auscultate fetal heart.

If on auscultation the fetal heart is abnormal, or the condition is Obstetric in nature, commence CTG, depending on gestational age. Escalate care to on call Obstetric Medical team and midwifery coordinator as per protocol.

If the woman is < 24 weeks gestation, bedside ultrasound may be needed to determine fetal wellbeing. CTG's performed at less than 28 weeks gestation should be reviewed by a Specialist Obstetrician

- Perform and record baseline maternal temperature, pulse, blood pressure and pain score. Determine MADDS score and escalate if required as per protocol.
- Perform an abdominal palpation, noting:
 - Any tenderness or rigidity
 - Fundal height
 - Fetal lie
 - Fetal presentation
 - Any uterine activity or irritability
- If there is any fresh vaginal bleeding, arrange urgent review by GPO, depending on the source of bleeding plan admission and try to organise an ultrasound. Manage APH according to the severity of bleeding, period of Gestation and source of bleeding.
- If there is any vaginal discharge (other than fresh bleeding) or uterine tenderness/irritability obtain woman's consent and perform a sterile speculum examination, noting cervical dilatation and type of discharge. Take HVS for MC&S (ensuring swabs are plated as well as placed in charcoal medium)
- If suspected TPL >24 weeks gestation, perform Partosure test and amniure if any suspicion of SROM.
- Collect MSU. If on dip stick there is >1+ proteinuria or if there are positive nitrates or leucocytes, send for MC&S
- If there are any signs of vaginal bleeding or infection i.e., maternal pyrexia or tachycardia then arrange for review by GPO and admit if necessary.
 - Obtain IV access.
 - Collect blood for FBP, G&H, CRP
 - Collect U&E and LFT and any necessary tests if indicated.
 - Perform blood culture if maternal temp is >38 degrees.
 - Start antibiotics if warranted and keep Specialist Obstetrician informed.
- Involve other specialities if obstetric conditions ruled out for the cause of abdominal pain.
- Diagnostic imaging should be performed as indicated since delay in diagnosis and treatment can increase maternal and fetal/newborn morbidity and mortality. Ultrasound for fetal wellbeing along with ultrasound for other areas e.g., KUB, Gall Bladder, should be performed as per the urgency of the case.
- If decision is made to discharge the woman, ensure she is aware of any follow up arrangements or when to return.

- Ensure all investigation results are followed up in a timely manner and management plan documented. Women need to be made aware of the plan and follow up accordingly.

5. Legislative context

The following documents are required to give effect to this policy:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Recognising and Responding to Acute Deterioration Policy MP 0171/22](#)
- HDWA [Cardiotocography Monitoring Policy MP 0076/18](#)
- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)
- EMHS [Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy EMHS: 195](#)

6. Supporting information

The following documents support the implementation of this policy:

- AKG [Discharge Guideline – Obstetrics Procedure](#)
- AKG [Diagnostic Results Management Policy](#)
- AKG [Recognition and Response to Acute Deterioration Policy](#)

7. Compliance, monitoring and evaluation.

Compliance against this guideline will be evaluated via:

- Monitoring of reporting clinical incident data via Datix CIMS.

Department specific findings are to be reported at the departmental meeting for review and identification of any opportunities for improvement.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.3 Partnering with consumers.
- Standard 5 – Action 5.11 Clinical assessment
- Standard 6 – Action 6.7 Clinical handover
- Standard 6 – Action 6.9 Communication critical information
- Standard 8 – Action 8.4 Recognising acute deterioration.
- Standard 8 – Action 8.6 Escalating care

9. References

References

1. UpToDate <https://www.uptodate-com.smhslibresources.health.wa.gov.au/contents/search>
2. Non-Specific Abdominal pain: Maternal Fetal Assessment Unit-Quick reference Guide: Complications of pregnancy Clinical Guidelines KEMH

10. Document owner

Enquiries relating to this guideline may be directed to:

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11. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	26/09/2024	26/09/2027	Scheduled review

12. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dr Tanis Strickland – A/Director of Clinical Services
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